

IMPLANT CHECK

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SAMPLE REPORT

Treatment Plan Review Report

CASE REF IC-2026-0008	SUBMITTED BY Dr Sarah Patel	REVIEWED BY Dr Avik Dandapat	DATE 25 June 2026
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APPROVED WITH RECOMMENDATIONS

The submitted plan is clinically reasonable based on the information provided. The recommendations below should be reviewed before proceeding to guided surgery or final restorative planning.

1

CASE SUMMARY

Review requested for replacement of the lower left first molar with a single implant-retained crown. The patient is medically stable, a non-smoker, and oral hygiene appears satisfactory. The submitted CBCT slice and intra-oral scan demonstrate a healed posterior mandibular ridge with adequate restorative space.

2

SAC CLASSIFICATION

Provisional ITI SAC classification: Straightforward to Advanced. The case is broadly suitable for a single posterior implant, but the proximity to the inferior alveolar canal and restorative emergence profile should be checked carefully during final planning.

3

IMPLANT POSITIONING

The proposed implant position is acceptable mesiodistally. Buccolingually, the fixture should be kept restorative-driven and centred under the planned crown rather than simply centred in the available bone. Maintain a minimum 1.5-2.0 mm buccal and lingual bone envelope where possible.

4

ANGULATION ANALYSIS

The planned angulation appears favourable for a screw-retained restoration. Confirm that the screw access emerges through the central fossa or slightly lingual to the occlusal table. Avoid a buccal screw-access trajectory, as this may compromise restoration design and cleansability.

5

ANATOMICAL CONSIDERATIONS

The available vertical height appears adequate, but final measurements should preserve a safety zone above the inferior alveolar canal. Check the mental foramen position on adjacent slices and confirm there is no undercut risk on the lingual plate.

6

PROSTHETIC CONSIDERATIONS

Restorative space is satisfactory for a molar crown. The emergence profile should be kept cleansable and not over-contoured. Consider a customised healing abutment or provisional contour if soft-tissue shaping is clinically important.

7

RISK ASSESSMENT

The main risks are anatomical safety, implant-restoration trajectory, and occlusal load management in the posterior mandible. Surgical risk: IAN safety margin and lingual concavity. Prosthetic risk: screw access and emergence profile. Patient risk: monitor parafunction and hygiene access.

8

RECOMMENDATIONS

Proceed only after confirming exact nerve clearance on the final planning software. Consider a surgical guide if there is any uncertainty regarding angulation or canal proximity. Document implant dimensions, safety margins, and restorative-driven position before surgery.

9

OVERALL VERDICT

The case is suitable to proceed, subject to the checks above. The submitted information supports a predictable single implant restoration, provided the final plan respects anatomical safety margins and prosthetically driven positioning.

Clinical disclaimer

This sample report is advisory and based only on the information submitted. The treating clinician remains responsible for diagnosis, consent, treatment planning, surgical execution, follow-up care and compliance with applicable clinical standards.

Dr Avik Dandapat

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